OPERATIONAL EXCELLENCE · WHAT THE RESEARCH SAYS

Many hospital teams now hold short safety huddles.

In one trial in four academic hospitals, adverse events fell by 12% after doctors started them.

But many huddles are not run as planned. Here is what five studies, published from 2021 to 2026, show.

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THE IDEA

What is a safety huddle?

A short, structured meeting, often held daily. The team names what could harm patients and agrees how to prevent it.

> An adverse event is harm caused by care itself, not by illness.

THE TORONTO TRIAL

The trial covered doctors in four academic hospitals

12%

drop in the rate of adverse events after doctors started huddles, based on the records of 912 patients.

64%

of doctors' answers on communication about error were positive, up from 36%.

Not every doctor attended: attendance ranged from 30% to 85% across hospitals.

THE UK PROJECT

Huddles are often run with parts missing

64

of 92 wards in five hospitals sustained the huddles.

4.9

of 9 intended elements were included in an average huddle.

Teams routinely left out two things: a chart showing harm over time, and a review of harm that had already happened.

WHO ATTENDS?

Most responding units said nurses attended regularly. Few said the same about doctors.

In a survey of 74 UK intensive care units, 71 said they held safety huddles.

89%

of responding units (63) reported that nurses attended regularly.

32%

of responding units (only 8) said the same about doctors.

The two percentages are based on different numbers of responding units.

Units listed drawbacks: huddles take time, can seem pointless, can interrupt patient care, and may lack psychological safety. That means staff feel they can speak up about a mistake without fear.

FOLLOW-UP

On one ward in Taiwan, every problem raised in the huddles was resolved

It was a medical ward, where 81 problems were raised in 44 huddles. Most were resolved within hours.

From the five studies, the author infers that a huddle is likely to help when decision-makers attend, risks can be raised without fear, and agreed actions are carried out.

The studies did not test this.

THE AUTHOR'S READING OF THE STUDIES

What can leaders do?

1

Give the huddle a clear purpose: what could harm patients today, and what will we do to prevent it?

2

Make sure doctors and other professional groups attend, not only nurses.

3

Measure two things: whether the huddle runs in full, and whether harm goes down.

All three steps are the author's inference, and no study tested step 3.

> **Three years from now, do you want your team's huddle to be a routine people forget, or the place where risks are caught early and the team agrees how to prevent harm?**

SOURCES AND LIMITS

What this evidence can and cannot tell us

The Toronto trial covered four academic hospitals and included only doctors.

The South Korean and Taiwanese studies had no comparison wards. The surveys measured staff views, not harm.

None of the five studies took place in an Arab hospital. Applying the idea to other sectors is an untested inference.

Editorial disclaimer: The views and analyses presented in this article represent the author's professional interpretation based on the cited sources and do not necessarily represent the official positions of the organizations mentioned. For the original, complete and most current information, please consult the primary sources and official documents listed in the references.

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